THE MATERNITY METHOD

NCLEX CHEAT SHEET

OB Medications

Maternal & Newborn › Pharmacology

The Big Picture OB meds cluster into a few jobs: **ripen and start labor**, **stop labor**, **protect baby's lungs and brain**, **control BP and seizures**, **stop bleeding**, and **protect the newborn**. Know each drug's **job**, its **big side effect**, and its **contraindication**.

KNOW THE NUMBERS

> 5 / 10 min	4–7 mEq/L	72 hrs	27–36 wks	4 wks
Contractions on **Pit** → stop it	Therapeutic **magnesium**	RhoGAM window **after birth**	**Tdap** every pregnancy	No pregnancy after **MMR**

Ripening & Induction

Drug	Watch for	Nurse must-know
Oxytocin (Pitocin)	**Tachysystole**, FHR changes, **water intoxication** (headache, confusion, low sodium)	IV **pump**, piggyback into the main line; **stop it** for tachysystole or bad strip
Dinoprostone (Cervidil insert / gel)	Tachysystole, nausea, fever	Lie down after insertion; **remove insert** for tachysystole; wait before starting Pit (per protocol)
Misoprostol (Cytotec)	Tachysystole, fever, diarrhea	**Avoid with prior cesarean** (rupture risk); wait about 4 hrs after last dose before Pit

Preterm Labor & Baby Protection

Drug	Watch for	Nurse must-know
Betamethasone	Mom's **blood sugar** rises	IM, **2 doses 24 hrs apart**, < 34 wks: matures **baby's lungs**
Magnesium sulfate	Flushing, drowsiness. **Toxic:** RR < 12, no DTRs, urine < 30 mL/hr	Seizure prevention + baby's brain < 32 wks. Antidote: **calcium gluconate**
Terbutaline	**Tachycardia**, tremor, high glucose, low K^+	Hold if mom's **HR > 120**; short-term only
Nifedipine	**Low BP**, headache, flushing	Check BP; caution with magnesium
Indomethacin	Early **ductus closure**, low fluid	**< 32 wks** only, about 48 hrs

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Preeclampsia BP Meds

Treat BP ≥ 160 / ≥ 110 within 30–60 min

Drug	Watch for	Nurse must-know
Labetalol (IV/PO)	**Low HR**, low BP	**Avoid in asthma** or heart block
Hydralazine (IV)	**Fast HR**, headache, low BP	Recheck BP often after each dose
Nifedipine (immediate-release PO)	Headache, flushing, low BP	Oral option when no IV

Stop the Bleeding (Uterotonics)

Drug	Watch for	Never give if…
Oxytocin	Low BP if pushed fast	**Never undiluted IV push**; 1st line, IV infusion or IM
Methylergonovine (Methergine) IM	**Raises BP**, nausea	**Hypertension or preeclampsia**
Carboprost (Hemabate) IM	**Diarrhea**, fever, vomiting	**Asthma** (causes bronchospasm)
Misoprostol (rectal/oral)	Fever, chills, diarrhea	Allergy
Tranexamic acid (IV)	Clot risk	Give **within 3 hrs** of birth



MEMORY HACK · UTEROTONIC CONTRAINDICATIONS

Methergine raises the BP · Hemabate tightens the airway

Methergine
check **BP first**: no HTN

Hemabate
check for **asthma**

Pitocin
never IV push

TXA
within **3 hours**

Other Must-Know Meds

Drug	Why	Nurse must-know
Rho(D) immune globulin (RhoGAM)	Stops Rh− mom from making antibodies	28 wks + **within 72 hrs** of Rh+ birth; IM; **blood product**
Opioids in labor (nalbuphine, butorphanol, fentanyl)	Pain relief	Newborn **resp depression** if given near birth; nalbuphine/butorphanol can cause **withdrawal** in opioid-dependent moms
Epidural	Labor pain	**Hypotension** → fluids, left tilt; **bladder** fills (catheter); fall risk
Naloxone	Reverses opioid resp depression	**Not** for babies of opioid-dependent moms (seizures)
Vaccines	Protect mom + baby	**Tdap** 27–36 wks; **flu** (inactivated) any time; **MMR/varicella** postpartum only (live)
Newborn: vitamin K, erythromycin, Hep B	Bleeding, eye infection, hepatitis B	Vitamin K in **vastus lateralis**; Hep B within 24 hrs

☆ MUST-KNOW FOR THE NCLEX

- **Oxytocin:** stop for tachysystole or non-reassuring FHR; watch for **water intoxication**.
- **Methergine → no hypertension. Hemabate → no asthma.**
- **Magnesium toxicity:** RR < 12, absent DTRs, low urine output → stop and give **calcium gluconate**.
- **Betamethasone** is for baby's lungs; check mom's glucose.
- **Live vaccines** (MMR, varicella) only **after** birth.

QUICK CHECK · TEST YOURSELF

1. A patient with preeclampsia is bleeding after birth. Which uterotonic is contraindicated?
2. Carboprost (Hemabate) should be avoided in patients with _____.
3. The antidote for magnesium sulfate is _____.
4. Oxytocin's antidiuretic effect can cause _____ _____.
5. Tdap is given between ____ and ____ weeks of every pregnancy.

Answers: 1. methylergonovine (Methergine) · 2. asthma · 3. calcium gluconate · 4. water intoxication · 5. 27 and 36

For study use only. Always follow facility protocols, current guidelines, and provider orders.

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